Supplemental Online Content

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eMethods

Patients

Eligible patients must have an Eastern Cooperative Oncology Group (ECOG) performance status score of 0 to 1, measurable disease according to the Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1, and adequate organ function. Main exclusion criteria included the presence of pancoast tumor, neuroendocrine carcinoma or sarcomatoid tumor, prior systemic anti-tumor therapy for early-stage NSCLC, active infection requiring systemic treatment, and a history of specified autoimmune diseases. The complete list of inclusion and exclusion criteria can be found in the study protocol.

Procedures

Patients underwent preoperative radiological evaluation 3-5 weeks after receipt of the last dose of neoadjuvant treatment. Patients who remained operable underwent radical resection as defined by the IASLC guideline 4-6 weeks after the last dose of neoadjuvant treatment. Resected tumor specimens were assessed by pathologists at the study site for surgical margins and extent of resection (R0, R1, or R2) and evaluation of pCR; tumor tissue was submitted to a central laboratory for blinded assessment of pathological response (MPR of pCR). Patients indicated for radiotherapy should start radiotherapy within 4 weeks after the end of adjuvant treatment and start maintenance treatment within 30 days (±3 days) after the end of radiotherapy. Tumor imaging must have included contrast-enhanced CT or MRI of the chest and of other anatomic sites with suspicious lesions (abdomen and pelvis shall be included at baseline), and subsequent radiological evaluations should include contrast-enhanced CT of the chest and PET-CT/CT or MRI scans of other anatomic sites with suspected recurrence.

AEs were recorded for patients who received any amount of study treatment up to 30 days (90 days for serious AEs) after the last dose of the study drug or the start of new anticancer therapy, whichever occurred first. After the discontinuation of the study treatment, all subjects were followed for survival, until death, withdrawal of consent, loss to follow-up, or termination of the study.

Statistical analysis methods

The stratified log-rank test would be used for the primary analysis for comparison of EFS between the two treatment groups. The HR for EFS and its 95% CI were to be estimated using the stratified Cox proportional hazards model. The Kaplan-Meier method was applied to estimate the median EFS for each treatment group. The Brookmeyer Crowley method, with log-log transformation applied to the survival function, was used to construct the 95% CI for the median EFS of each treatment group. The methods outlined for EFS analyses were also used for OS and DFS analyses.

The stratified Cochran Mantel Haenszel (CMH) method would be used for the analysis comparison of MPR and pCR between the two treatment groups. The Mantel Haenszel (MH) method would be used to calculate the difference in rate between the two treatment groups and its 95% CI (treatment group minus control group).

The efficacy population for the interim EFS analysis included all randomized stage III patients (n=404). The safety population for the interim analysis included all randomized Stage III patients who received any amount of the study drug (n=404). The details of statistical analyses are available in the statistical analysis plan.

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eTable 1. Reasons for not meeting eligibility criteria

Code	Inclusion or exclusion criteria details	N
INCL03	Treatment-naïve, histologically confirmed, resectable, Stage IIIA, IIIB (N2) NSCLC (AJCC Cancer Staging Manual, version 8). The clinical tumor, node, and metastasis (cTNM) stage can be confirmed through positron emission tomography-computed tomography (PET-CT) or by pathological biopsy. Resectability is defined as resectable or potentially resectable according to the multidisciplinary consensus statement on the clinical management of subjects with stage III non-small cell lung cancer. Resectable NSCLC includes any of the following cTNM stages: • Stage II is defined as radically resectable, as assessed by a qualified thoracic surgeon • Stage IIIA (N0-N1), some N2 with single-station mediastinal lymph node < 2 cm in shortest diameter, and some T4 (satellite nodules present in adjacent lobes) N1 Potentially resectable NSCLC includes the following cTNM stages: • Stage IIIA and IIIB based on ipsilateral mediastinal nodes (N2) less than 3 cm in shortest diameter • Stage III with T3 or T4 centrally located tumors determined to be potentially resectable by a surgeon Pathological puncture verification is strongly recommended for suspicious lesions detected by imaging studies that are presenting as clinically normal, which can lead to TNM stage changes in areas including but not limited to contralateral mediastinal lymph nodes, supraclavicular lymph nodes, solid/subsolid pulmonary nodule(s), and non-simple ground glass opacity (GGO).	251
EXCL03	Participants with known active EGFR-sensitive mutations or ALK translocation. EGFR and ALK mutation status must be identified for the subjects with non-squamous cell carcinoma.	100
EXCL01	Presence of locally advanced, unresectable, or metastatic disease. Unresectable disease is defined by the following criteria outlined in the multidisciplinary consensus statement on the clinical management of subjects with stage III non-small cell lung cancer: Stage IIIC, Stage IIIB with N3 lymph node involvement, Stage IIIA or IIIB with N2 involvement of single-station mediastinal lymph node where the shortest diameter is ≥3 cm, N2 involvement characterized by the fusion of multi-station lymph node where the shortest diameter is ≥2 cm on CT, T4 with invasion of the esophagus, heart, aorta, or other adjacent tissues, which in the opinion of the investigator, renders the subject unresectable.	40
INCL08	Being willing and able to comply with the visits, treatment plan, laboratory examinations, and other study procedures scheduled in the study.	18
EXCL09	Uncontrolled active hepatitis B infection, defined as positive hepatitis B surface antigen [HBsAg] with detectable hepatitis B virus-deoxyribonucleic acid (HBV-DNA) above the ULN at the clinical laboratory of the study center, or active hepatitis C infection, defined as hepatitis C surface antibody seropositive and hepatitis C virus-ribonucleic acid (HCV-RNA) during screening period. Subjects with HBV-DNA <500 IU/mL within 28 days prior to randomization, who received antiviral therapy per local standard of care for at least 14 days and commit to take antiviral therapy continuously during the study, are eligible.	17
EXCL17	History or current evidence of any disease, concomitant medication, or abnormal laboratory	17

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Code	Inclusion or exclusion criteria details	N
	results that may confound the safety or efficacy assessment, interfere with the subject's participation throughout the full course of the study, or prevent the subject from receiving the maximum benefit from participating in the study, as judged by the investigators;	
INCL07	Adequate organ function: • Hematology: Absolute neutrophil count (ANC) ≥1500/μL; platelet ≥100000/μL; hemoglobin ≥9.0 g/dL. • Renal function: Serum creatinine ≤1.5× upper limit of normal (ULN) or calculated creatinine clearance (CrCl) ≥60 mL/min (using Cockcroft-Gault formula). • Hepatic function: Total bilirubin ≤1.5× ULN or direct bilirubin within the normal range for subjects with a total bilirubin level >1.5× ULN; aspartate aminotransferase (AST) and alanine aminotransferase (ALT) ≤2.5× ULN. • Endocrine function: Thyroid-stimulating hormone (TSH) within the normal range. Note: subjects may still meet the inclusion criteria if the TSH levels are outside the normal range at baseline, as long as T3 and free T4 are within the normal ranges. • Coagulation function: International normalized ratio (INR) or prothrombin time (PT), as well as activated partial thromboplastin time (aPTT) ≤1.5× ULN.	10
INCL09	Total lung function is able to withstand the proposed lung resection procedure as assessed by a surgeon.	10
INCL10	Women of childbearing potential must undergo a serum pregnancy test within 3 days prior to the first dose of the investigational product, and the result must be negative. Female subjects of childbearing potential and male subjects whose partners are women of childbearing potential must agree to use highly effective contraceptive methods during the study period and for at least 180 days after the last dose of the study drug.	6
INCL01	Have sufficient understanding of this study and voluntarily sign the Informed Consent Form (ICF).	5
EXCL02	Pancoast tumor, or in cases where the tumor histology corresponds to a large cell neuroendocrine carcinoma or sarcomatoid tumor.	5
INCL05	Tumor tissue specimens should be provided for pathological evaluation to confirm the diagnosis and for the detection of PD-L1 tumor expression and biomarkers prior to randomization. Tumor tissue specimens can be freshly obtained, while archived samples obtained within 3 months prior to enrollment. Acceptable tumor tissue specimens must contain tumor cells and be acquired through core needle biopsy, excisional biopsy during bronchoscopy, or surgical resection. Tumor samples acquired through fine-needle aspiration or bronchial brushing are not acceptable.	4
EXCL07	Active infections requiring systemic treatment.	3
EXCL18	Presence of any malignancy other than non-small cell lung cancer within 5 years prior to the first dose. Exception: subjects with a history of a malignancy with negligible risk of metastasis or death (e.g., expected disease-free survival >5 years) and anticipate a complete cure after definitive treatment (e.g., appropriately treated carcinoma in situ of the cervix, basal or squamous cell skin cancer, or ductal breast carcinoma in situ treated) are eligible.	2
INCL02	Aged 18 to 70 years old, male or female.	1
EXCL04	Previous systemic anti-tumor therapy, including investigational products, for early-stage	1

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Code	Inclusion or exclusion criteria details	N
	NSCLC.	
EXCL06	History of active tuberculosis.	1
EXCL08	Subjects with any known or suspected autoimmune disease or immunodeficiency, with the following exceptions: patients with a history of hypothyroidism, if hormone replacement therapy is not needed or currently stable while receiving hormone replacement therapy at a physiological dose; subjects with stable type I diabetes whose blood glucose is controlled.	1

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eTable 2. Summary of systemic treatments

		Toripalimab + chemo n = 202	Placebo + chemo n = 202
Neoadjuvant*	**Received treatment, n (%)**	202 (100)	202 (100)
	Completed 1 cycles, n (%)	2 (1.0)	5 (2.5)
	Completed 2 cycles, n (%)	10 (5.0)	5 (2.5)
	Completed 3 cycles, n (%)	176 (87.1)	185 (91.6)
	Completed 4 cycles, n (%)	14 (6.9)	7 (3.5)
	Cisplatin-based therapy, n (%)	37[#] (18.3)	31[^] (15.3)
	Carboplatin-based therapy, n (%)	166[#] (82.2)	170[^] (84.2)
Adjuvant	**Received treatment, n (%)**	144 (71.3)	131 (64.9)
Maintenance	**Received treatment, n (%)**	145 (71.8)	130 (64.4)
	Ongoing treatment, n (%)	25 (12.4)	20 (9.9)
	Discontinued treatment, n (%)	120 (59.4)	110 (54.5)
	Completed 13 cycle, n (%)	88 (43.6)	66 (32.7)

* 3 cycles of neoadjuvant chemotherapy with 4 cycles of peri-operative chemotherapy in total were required in Neotorch study. Surgeons were allowed to determine the most appropriate timing for surgery based on the patient's condition.

One patient in toripalimab group received both cisplatin and carboplatin treatment.

^ One patient in placebo group was unable to complete chemotherapy due to allergy to paclitaxel and received neither cisplatin nor carboplatin treatment.

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eTable 3. Summary of surgery

	Toripalimab + chemo n = 202	Placebo + chemo n = 202
No surgery performed, n (%)	36 (17.8)	54 (26.7)
Disease progression	5(2.5)	31(15.3)
Patient refusal	18 (8.9)	13 (6.4)
Adverse event[#]	6 (3.0)	0
Other *	7 (3.5)	10 (5.0)
Patient underwent surgery, n (%)	166 (82.2)	148 (73.3)
R0 resection n (% of underwent surgery)	159 (95.8)	137 (92.6)
Type of surgery (% of underwent surgery)		
Lobectomy	134 (80.7)	123 (83.1)
Sleeve lobectomy	15 (9.0)	11 (7.5)
Pneumonectomy	15(9.0)	14 (9.5)
Regional lymph node dissection only	2 (1.2)	0

[#] In toripalimab group, 6 patients had no surgery performed due to AEs. Three AEs were not related to toripalimab, including aggravation of cirrhosis, pneumonia and cerebral infarction as assessed by investigators. The other three AEs were related to toripalimab, including immune-mediated hepatitis, hypophysitis and pneumonitis as assessed by investigators.

* Other reasons included unresectable at the baseline in 3 patients in toripalimab group and 5 in placebo group, unresectable during intraoperative exploration in 1 patient in toripalimab group and 2 in placebo group, unfit for surgery due to poor lung function in 1 patients in toripalimab group and intolerant to anesthetic in 1 patient in placebo group, high risk of surgery due to tumor enclosing the blood vessels or unremarkable shrinkage in each patient in toripalimab group, and high risk of surgery due to lower limbs edema and ventricular premature and lost to follow up in each patient in placebo group.

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eTable 4. Subsequent anti-cancer therapy after the study treatment in stage III NSCLC patients

	Toripalimab + chemo	**Placebo + chemo**
	n = 202	**n = 202**
New anti-cancer systemic therapy	47 (23.3)	77 (38.1)
PD-1/L1 Inhibitors	21 (10.4)	51 (25.2)
Tislelizumab	8 (4.0)	19 (9.4)
Camrelizumab	7 (3.5)	3 (1.5)
Sintilimab	4 (2.0)	19 (9.4)
Toripalimab	3 (1.5)	5 (2.5)
Pembrolizumab	1 (0.5)	4 (2.0)
Envafolimab	1 (0.5)	0
Durvalumab	0	2 (1.0)
Nivolumab	0	2 (1.0)
Atezolizumab	0	1 (0.5)
Cytotoxic Therapy	42 (20.8)	70 (34.7)
EGFR-TKIs	5 (2.5)	3 (1.5)
Antiangiogenic therapy	15 (7.4)	25 (12.4)
Other	2 (1.0)	3 (1.5)
Radiotherapy	24 (11.9)	33 (16.3)
Surgical treatment	1 (0.5)	7 (3.5)
Other anti-cancer therapy	25 (12.4)	23 (11.4)

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eTable 5. Overall survival in stage III NSCLC patients

	Toripalimab +chemotherapy (N=202)	**Placebo + chemotherapy (N=202)**
Status, n (%)		
Death	28 (13.9)	42 (20.8)
Censored	174 (86.1)	160 (79.2)
Estimate of median OS (months)	NE	30.4
(95% CI) [1]	(NE, NE)	(29.2, NE)
Stratified analyses [2]		
Estimate of HR (toripalimab vs placebo) [3] (95% CI)	0.62 (0.381, 0.999)	
Log-rank p-value (two-sided)	0.0502	
Unstratified analyses		
Estimate of HR (toripalimab vs placebo) [3] (95% CI)	0.62 (0.383, 1.001)	
Log-rank p-value (two-sided)	0.0511	
1-Year OS Rate	94.4%	89.6%
(95% CI) [4]	(90.07, 96.85)	(84.31, 93.15)
Difference of 1-Year OS Rates	4.8%	
(95% CI) [5]	(-3.5, 11.1)	
2-Year OS Rate	81.2%	74.3%
(95% CI) [4]	(72.87, 87.19)	(65.86, 80.92)
Difference of 2-Year OS Rates	6.9%	
(95% CI) [5]	(2.3, 23.4)	

NE, not estimable.

Note: OS is defined as the time from randomization to death from any cause and is calculated as (death date or censoring date - randomization date + 1)/30.4375 in month.

[1] The Brookmeyer Crowley methodology was used to construct the 95% CI for the median overall survival. [2] The stratified analyses used ECOG performance status (0 vs. 1), and disease stage (recurrent vs. metastatic) as recorded in the IWRS.

[3] The hazard ratio was estimated with the use of the Cox proportional hazards model. Exact method was used to handle ties.

[4] The 95% CI was estimated with use of the standard error derived from Greenwood's formula.

[5] The 95% CI was estimated with use of the normal approximation method.

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eTable 6. Summary of study drug exposure

	Toripalimab +chemotherapy (N=202)	Placebo +chemotherapy (N=202)
The number of toripalimab/Placebo cycles		
Mean	11.4	10.1
Sd	6.00	6.17
Median	13.5	11.0
Q1, Q3	4, 17	3, 17
Weeks of exposure for toripalimab/Placebo		
Mean	36.74	32.31
Sd	19.594	20.467
Median	46.57	32.86
Q1, Q3	14.14, 51.86	9.29, 51.43
The number of cycles neoadjuvant therapy received		
Mean	3.0	3.0
Sd	0.40	0.40
Median	3.0	3.0
Q1, Q3	3, 3	3, 3
Weeks of exposure for neoadjuvant therapy		
Mean	9.44	9.13
Sd	2.057	1.532
Median	9.00	9.00
Q1, Q3	9.00, 9.29	9.00, 9.29
1 cycle of neoadjuvant treatment, n (%)	2 (1.0)	5 (2.5)
2 cycles of neoadjuvant treatment, n (%)	10 (5.0)	5 (2.5)
3 cycles of neoadjuvant treatment, n (%)	176 (87.1)	185 (91.6)
4 cycles of neoadjuvant treatment, n (%)	14 (6.9)	7 (3.5)
The number of cycles adjuvant therapy received		
Mean	1.0	1.0
Sd	0.20	0.09
Median	1.0	1.0
Q1, Q3	1, 1	1, 1
Weeks of exposure for adjuvant therapy		
Mean	3.04	3.02
Sd	0.653	0.607

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	Toripalimab +chemotherapy (N=202)	Placebo +chemotherapy (N=202)
Median	3.00	3.00
Q1, Q3	3.00, 3.00	3.00, 3.00
The number of cycles maintenance therapy received		
Mean	10.7	10.1
Sd	3.50	3.75
Median	13.0	13.0
Q1, Q3	9, 13	7, 13
Weeks of exposure for maintenance therapy		
Mean	35.01	32.97
Sd	12.140	13.583
Median	39.00	39.00
Q1, Q3	27.43, 40.43	21.57, 40.14

Sd, standard deviation.

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eTable 7. Summary of treatment emergent adverse events in stage III patients

Adverse Event Category	Toripalimab + Chemotherapy N=202 (%)	Placebo + Chemotherapy N=202(%)
Any TEAE	201 (99.5)	199 (98.5)
Any TEAE Grade ≥3	128 (63.4)	109 (54.0)
Any SAE	82 (40.6)	57 (28.2)
Any SAE Grade ≥3	63 (31.2)	51 (25.2)
Any TEAEs resulting in death	6 (3.0)	4 (2.0)
Any TEAE leading to interruption of toripalimab/placebo	57 (28.2)	29 (14.4)
Any TEAE results in permanent discontinuation of toripalimab/placebo	19 (9.4)	15 (7.4)
Any Investigator-determined irAE	85 (42.1)	46 (22.8)
Any Investigator-determined irAE with Grade ≥3	24 (11.9)	6 (3.0)
Any infusion-related reaction	7 (3.5)	13 (6.4)

TEAE: treatment-emergent adverse event, SAE: serious adverse event, irAE: immune-related adverse event.

Note: TEAE is defined as all adverse events that newly occur or get worsened from baseline during the period from the first dose to 90 days after the last dose or the start of new anti-tumor therapy, whichever occurs first.

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eTable 8. Fatal adverse events

	Toripalimab + chemotherapy N = 202 n (%)		**placebo + chemotherapy N = 202 n (%)**	
System Organ Class Prefer term	TEAE	TEAE related to toripalimab	TEAE	TEAE related to placebo
Overall	6 (3.0)	0	4 (2.0)	0
Cerebral hemorrhage	1 (0.5)	0	0	0
Cerebral infarction	1 (0.5)	0	0	0
Malignant neoplasm progression	2 (1.0)	0	1 (0.5)	0
Pneumonitis	1 (0.5)	1 (0.5)	0	0
Respiratory failure	0	0	1 (0.5)	0
Bronchopleural fistula	0	0	1 (0.5)	0
Pneumonia	1 (0.5)	0	0	0
Infectious pleural effusion	0	0	1 (0.5)	0

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eTable 9. Treatment-emergent adverse events reported in at least 5% of the patients in either treatment group

Preferred Term	Toripalimab + Chemo (N=202)	Placebo + Chemo (N=202)
Any TEAE	201 (99.5)	199 (98.5)
Anemia	133 (65.8)	132 (65.3)
Neutropenia	114 (56.4)	106 (52.5)
Leukopenia	110 (54.5)	97 (48.0)
Cough	69 (34.2)	47 (23.3)
Alanine aminotransferase increased	66 (32.7)	47 (23.3)
Aspartate aminotransferase increased	64 (31.7)	40 (19.8)
Hypoproteinemia	60 (29.7)	53 (26.2)
Arrhythmia	57 (28.2)	51 (25.2)
Alopecia	56 (27.7)	60 (29.7)
Decreased appetite	56 (27.7)	29 (14.4)
Nausea	54 (26.7)	36 (17.8)
Thrombocytopenia	53 (26.2)	45 (22.3)
Pneumonia	50 (24.8)	43 (21.3)
Incision site pain	47 (23.3)	37 (18.3)
Fatigue	45 (22.3)	31 (15.3)
Constipation	41 (20.3)	23 (11.4)
Rash	41 (20.3)	19 (9.4)
Neuropathy peripheral	36 (17.8)	29 (14.4)
Pyrexia	35 (17.3)	31 (15.3)
Hypothyroidism	34 (16.8)	7 (3.5)
Hyperglycemia	33 (16.3)	28 (13.9)
White blood cell increased	33 (16.3)	22 (10.9)
Hyperuricemia	32 (15.8)	20 (9.9)
Wound complication	32 (15.8)	32 (15.8)
Non-cardiac chest pain	31 (15.3)	24 (11.9)
Weight increased	31 (15.3)	25 (12.4)
Neutrophil count increased	30 (14.9)	21 (10.4)
Pneumonitis	30 (14.9)	17 (8.4)
Renal function test abnormal	30 (14.9)	22 (10.9)
Vomiting	30 (14.9)	27 (13.4)
Diarrhea	29 (14.4)	12 (5.9)
Lymphopenia	29 (14.4)	19 (9.4)
Pulmonary function test decreased	27 (13.4)	24 (11.9)
Hypokalemia	26 (12.9)	16 (7.9)
Hyperthyroidism	25 (12.4)	8 (4.0)
Gamma-glutamyl transferase increased	24 (11.9)	13 (6.4)

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Preferred Term	Toripalimab + Chemo (N=202)	Placebo + Chemo (N=202)
Weight decreased	24 (11.9)	17 (8.4)
Arthritis	23 (11.4)	22 (10.9)
Hyperbilirubinemia	23 (11.4)	21 (10.4)
Blood lactate dehydrogenase increased	22 (10.9)	10 (5.0)
Musculoskeletal pain	22 (10.9)	26 (12.9)
Upper respiratory tract infection	22 (10.9)	16 (7.9)
Dyspnea	21 (10.4)	5 (2.5)
Hyponatremia	21 (10.4)	17 (8.4)
Hepatic function abnormal	20 (9.9)	14 (6.9)
Insomnia	20 (9.9)	12 (5.9)
Blood alkaline phosphatase increased	18 (8.9)	13 (6.4)
Blood creatine phosphokinase increased	18 (8.9)	13 (6.4)
Hemoptysis	18 (8.9)	17 (8.4)
Hyperlipidemia	18 (8.9)	25 (12.4)
Thyroid function test abnormal	18 (8.9)	20 (9.9)
Pruritus	17 (8.4)	8 (4.0)
Hematuria	16 (7.9)	17 (8.4)
Blood electrolytes abnormal	14 (6.9)	16 (7.9)
Pain	14 (6.9)	14 (6.9)
Stomatitis	14 (6.9)	9 (4.5)
Abdominal distension	13 (6.4)	8 (4.0)
Dizziness	13 (6.4)	11 (5.4)
Hypertension	13 (6.4)	10 (5.0)
Pleural effusion	13 (6.4)	4 (2.0)
Proteinuria	13 (6.4)	12 (5.9)
Procedural pain	12 (5.9)	13 (6.4)
Restrictive pulmonary disease	12 (5.9)	10 (5.0)
Urinary tract infection	12 (5.9)	7 (3.5)
Fibrin D dimer increased	11 (5.4)	11 (5.4)
Hypocalcemia	11 (5.4)	3 (1.5)
Abdominal pain	10 (5.0)	6 (3.0)
Blood creatine phosphokinase MB increased	10 (5.0)	4 (2.0)
Hiccups	7 (3.5)	14 (6.9)

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eTable 10. Grade 3 or higher treatment-emergent adverse events reported in at least two patients in either treatment group

Preferred Term	Toripalimab + Chemo (N=202)	Placebo + Chemo (N=202)
Any TEAE with Grade ≥3	128 (63.4)	109 (54.0)
Neutropenia	68 (33.7)	60 (29.7)
Leukopenia	30 (14.9)	16 (7.9)
Pneumonia	25 (12.4)	21 (10.4)
Anemia	18 (8.9)	16 (7.9)
Thrombocytopenia	14 (6.9)	7 (3.5)
Lymphopenia	8 (4.0)	8 (4.0)
Pneumonitis	8 (4.0)	2 (1.0)
Hyperglycemia	6 (3.0)	1 (0.5)
Hypertension	6 (3.0)	3 (1.5)
Hypokalemia	6 (3.0)	1 (0.5)
Hepatic function abnormal	5 (2.5)	2 (1.0)
Rash	5 (2.5)	0
Weight increased	5 (2.5)	0
Alanine aminotransferase increased	4 (2.0)	1 (0.5)
Vomiting	4 (2.0)	0
Arrhythmia	3 (1.5)	2 (1.0)
Blood creatine phosphokinase increased	3 (1.5)	2 (1.0)
Hemoptysis	3 (1.5)	0
Hypersensitivity	3 (1.5)	3 (1.5)
Incision site pain	3 (1.5)	1 (0.5)
Myelosuppression	3 (1.5)	1 (0.5)
Aspartate aminotransferase increased	2 (1.0)	0
Bronchopleural fistula	2 (1.0)	1 (0.5)
Cerebral infarction	2 (1.0)	2 (1.0)
Diarrhea	2 (1.0)	0
Gamma-glutamyl transferase increased	2 (1.0)	1 (0.5)
Hyperlipidemia	2 (1.0)	2 (1.0)
Malignant neoplasm progression	2 (1.0)	2 (1.0)
Nausea	2 (1.0)	0
Pain	2 (1.0)	1 (0.5)
Pleural infection	2 (1.0)	0
Post procedural infection	2 (1.0)	2 (1.0)
White blood cell increased	2 (1.0)	1 (0.5)
Blood electrolytes abnormal	1 (0.5)	3 (1.5)
Fatigue	1 (0.5)	2 (1.0)
Hyperbilirubinemia	1 (0.5)	2 (1.0)

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Preferred Term	Toripalimab + Chemo (N=202)	Placebo + Chemo (N=202)
Non-cardiac chest pain	1 (0.5)	2 (1.0)
Respiratory failure	1 (0.5)	2 (1.0)
Pneumothorax	0	3 (1.5)
Pulmonary embolism	0	2 (1.0)

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eTable 11. Immune related adverse events (irAE) reported in more than one patient in either group as assessed by the investigator

No. of patients (%)	Toripalimab + chemo N=202 (%)		Placebo + chemo N=202 (%)	
	All Grades	Grades ≥3	All Grades	Grade ≥3
Any investigator-determined irAE	85 (42.1)	24 (11.9)	46 (22.8)	6 (3.0)
Hypothyroidism	29 (14.4)	0	6 (3.0)	0
Hyperthyroidism	21 (10.4)	0	3 (1.5)	0
Rash	21 (10.4)	2 (1.0)	7 (3.5)	0
Thyroid function test abnormal	13 (6.4)	0	9 (4.5)	0
Pneumonitis	12 (5.9)	5 (2.5)	2 (1.0)	1 (0.5)
Aspartate aminotransferase increased	8 (4.0)	0	3 (1.5)	0
Renal function test abnormal	8 (4.0)	0	3 (1.5)	0
Alanine aminotransferase increased	6 (3.0)	0	4 (2.0)	1 (0.5)
Blood creatine phosphokinase increased	6 (3.0)	2 (1.0)	1 (0.5)	0
Pruritus	5 (2.5)	0	1 (0.5)	0
Adrenal insufficiency	4 (2.0)	1 (0.5)	0	0
Gamma-glutamyl transferase increased	4 (2.0)	0	2 (1.0)	0
Hyperlipidemia	4 (2.0)	0	1 (0.5)	0
Hypokalemia	4 (2.0)	2 (1.0)	0	0
Alpha hydroxybutyrate dehydrogenase increased	3 (1.5)	0	0	0
Anemia	3 (1.5)	0	5 (2.5)	1 (0.5)
Blood creatine phosphokinase MB increased	3 (1.5)	0	0	0
Blood lactate dehydrogenase increased	3 (1.5)	0	0	0
Electrocardiogram T wave abnormal	3 (1.5)	0	1 (0.5)	0
Hepatic function abnormal	3 (1.5)	3 (1.5)	4 (2.0)	1 (0.5)
Hepatitis	3 (1.5)	0	2 (1.0)	0
Hyperbilirubinemia	3 (1.5)	0	6 (3.0)	2 (1.0)
Hyperglycemia	3 (1.5)	2 (1.0)	4 (2.0)	0
Hypoproteinemia	3 (1.5)	0	2 (1.0)	0
Immune-mediated thyroiditis	3 (1.5)	0	0	0
Myocarditis	3 (1.5)	0	0	0
Nausea	3 (1.5)	0	1 (0.5)	0
Neutropenia	3 (1.5)	1 (0.5)	2 (1.0)	1 (0.5)
Pyrexia	3 (1.5)	0	0	0
Thrombocytopenia	3 (1.5)	0	4 (2.0)	0
Abdominal distension	2 (1.0)	0	1 (0.5)	0
Abdominal pain	2 (1.0)	0	0	0
Diarrhea	2 (1.0)	0	1 (0.5)	0
Enteritis	2 (1.0)	0	0	0
Eye pain	2 (1.0)	0	0	0

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No. of patients (%)	Toripalimab + chemo N=202 (%)		Placebo + chemo N=202 (%)	
	All Grades	Grades ≥3	All Grades	Grade ≥3
Hypophysitis	2 (1.0)	1 (0.5)	0	0
Leukopenia	2 (1.0)	0	3 (1.5)	1 (0.5)
Neuropathy peripheral	2 (1.0)	0	1 (0.5)	0
Pneumonia	2 (1.0)	2 (1.0)	0	0
Stomatitis	2 (1.0)	1 (0.5)	0	0
Thyroid disorder	2 (1.0)	0	2 (1.0)	0
Proteinuria	1 (0.5)	0	2 (1.0)	0
Decreased appetite	1 (0.5)	0	2 (1.0)	0
Fatigue	1 (0.5)	1 (0.5)	2 (1.0)	0
Hyperuricemia	1 (0.5)	0	2 (1.0)	0

Note: For the same patient if the same adverse event occurs multiple times, that patient is counted only once in the same system organ classification and preferred terminology when calculating the number of adverse events.

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Table 12. Grade 3 or higher immune related adverse events (irAE) reported in either group as assessed by the investigator

Preferred Term	Toripalimab + Chemo (N=202)	Placebo + Chemo (N=202)
Any investigator-determined irAE with grade ≥3	24 (11.9)	6 (3.0)
Pneumonitis	5 (2.5)	1 (0.5)
Hepatic function abnormal	3 (1.5)	1 (0.5)
Blood creatine phosphokinase increased	2 (1.0)	0
Hyperglycemia	2 (1.0)	0
Hypokalemia	2 (1.0)	0
Pneumonia	2 (1.0)	0
Rash	2 (1.0)	0
Adrenal insufficiency	1 (0.5)	0
Arthritis	1 (0.5)	0
Fatigue	1 (0.5)	0
Herpes zoster	1 (0.5)	0
Hypersensitivity	1 (0.5)	0
Hypophysitis	1 (0.5)	0
Immune-mediated hypophysitis	1 (0.5)	0
Musculoskeletal pain	1 (0.5)	0
Neutropenia	1 (0.5)	1 (0.5)
Rash papular	1 (0.5)	0
Stomatitis	1 (0.5)	0
Alanine aminotransferase increased	0	1 (0.5)
Anemia	0	1 (0.5)
Hyperbilirubinemia	0	2 (1.0)
Leukopenia	0	1 (0.5)
Lymphopenia	0	1 (0.5)

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eTable 13. Common treatment-emergent adverse events that reported in at least 10% of the patients in either treatment group during the neoadjuvant treatment phase

System Organ Class	Toripalimab + Chemotherapy N = 202		Placebo +Chemotherapy N= 202	
Preferred Term	**All Grades**	**Grades ≥3**	**All Grades**	**Grade ≥3**
Any TEAE, n (%)				
Anemia	128(63.4)	16(7.9)	126(62.4)	15(7.4)
Neutropenia	100(63.4)	59(29.2)	92(46.0)	51(25.2)
Leukopenia	95(47.0)	23(11.4)	86(42.6)	13(6.4)
Alopecia	56(27.7)	0	60(29.7)	0
Alanine aminotransferase increased	54(26.7)	4(2.0)	44(21.8)	1(0.5)
Hypoproteinemia	54(26.7)	0	52(25.7)	0
Cough	54(26.7)	1(0.5)	36(17.8)	1(0.5)
Aspartate aminotransferase increased	52(25.7)	1(0.5)	37(18.3)	0
Nausea	47(23.3)	1(0.5)	28(13.9)	0
Incision site pain	47(23.3)	2(1.0)	36(17.8)	1(0.5)
Thrombocytopenia	46(22.8)	12(5.9)	39(19.3)	6(3.0)
Arrhythmia	46(22.8)	3(1.5)	33(16.3)	0
Pneumonia	43(21.3)	20(9.9)	36(17.8)	18(8.9)
Constipation	39(19.3)	0	22(10.9)	0
Decreased appetite	38(18.8)	0	26(12.9)	0
Fatigue	35(17.3)	0	28(13.9)	2(1.0)
White blood cell increased	32(15.8)	2(1.0)	21(10.4)	1(0.5)
Neuropathy peripheral	32(15.8)	1(0.5)	27(13.4)	1(0.5)
Wound complication	31(15.3)	0	32(15.8)	0
Neutrophil count increased	29(14.4)	0	20(9.9)	1(0.5)
Pyrexia	28(13.9)	0	28(13.9)	1(0.5)
Pulmonary function test decreased	26(12.9)	0	23(11.4)	1(0.5)
Weight increased	25(12.4)	4(2.0)	12(5.9)	0
Rash	25(12.4)	5(2.5)	16(7.9)	0
Vomiting	24(11.9)	3(1.5)	21(10.4)	0
Lymphopenia	24(11.9)	7(3.5)	17(8.4)	6(3.0)
Hypothyroidism	23(11.4)	0	3(1.5)	0
Hyperglycemia	23(11.4)	4(2.0)	19(9.4)	1(0.5)
Hyperuricemia	21(10.4)	1(0.5)	15(7.4)	0
Gamma-glutamyl transferase increased	21(10.4)	1(0.5)	11(5.4)	1(0.5)
Non-cardiac chest pain	21(10.4)	0	18(8.9)	1(0.5)
Musculoskeletal pain	18(8.9)	1(0.5)	23(11.4)	1(0.5)
Diarrhea	9(4.5)	2(1.0)	10(5.0)	0

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eTable 14. Common surgery-related postoperative adverse events that reported in at least 10% of the patients in either treatment group

System Organ Class Preferred Term	**Toripalimab + Chemotherapy N = 166**		**Placebo + Chemotherapy N= 148**	
	All Grades	**Grades ≥3**	**All Grades**	**Grade ≥3**
Any TEAE, n (%)				
Anaemia	44(26.5)	6(3.6)	39(26.4)	7(4.7)
Incision site pain	35(21.1)	3(1.8)	29(19.6)	1(0.7)
Wound complication	29(17.5)	0	32(21.6)	0
Pneumonia	27(16.3)	16(9.6)	22(14.9)	12(8.1)
Hypoproteinaemia	25(15.1)	0	28(18.9)	0
Cough	22(13.3)	0	20(13.5)	1(0.7)
Neutrophil count increased	22(13.3)	0	15(10.1)	0
White blood cell increased	22(13.3)	1(0.6)	16(10.8)	1(0.7)

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eTable 15. Common treatment-emergent adverse events that reported in at least 10% of the patients in either treatment group during the chemo-immunotherapy adjuvant treatment phase

System Organ Class Preferred Term	**Toripalimab + Chemotherapy N = 153**		**Placebo + Chemotherapy N= 134**	
	All Grades	**Grades ≥3**	**All Grades**	**Grade ≥3**
Any TEAE, n (%)				
Neutropenia	37(24.2)	18(11.8)	33(24.6)	18(13.4)
Leukopenia	37(24.2)	7(4.6)	34(25.4)	6(4.5)
Anaemia	26(17.0)	2(1.3)	28(20.9)	4(3.0)
Nausea	17(11.1)	1(0.7)	11(8.2)	0
Thrombocytopenia	15(9.8)	5(3.3)	17(12.7)	2(1.5)

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eTable 16. Common treatment-emergent adverse events that reported in at least 10% of the patients in either treatment group during the monotherapy maintenance treatment phase

System Organ Class	Toripalimab N = 145		Placebo N= 130	
Preferred Term	**All Grades**	**Grades ≥3**	**All Grades**	**Grade ≥3**
Any TEAE, n (%)				
Anemia	32(22.1)	3(2.1)	22(16.9)	1(0.8)
Alanine aminotransferase increased	23(15.9)	0	10(7.7)	0
Aspartate aminotransferase increased	22(15.2)	1(0.7)	8(6.2)	0
Cough	21(14.5)	0	12(9.2)	0
Hyperuricemia	20(13.8)	0	10(7.7)	0
Hypothyroidism	19(13.1)	0	4(3.1)	0
Neutropenia	18(12.4)	1(0.7)	18(13.8)	2(1.5)
Leukopenia	11(7.6)	1(0.7)	13(10.0)	0
White blood cell increased	18(12.4)	0	12(9.2)	0
Arrhythmia	17(11.7)	0	24(18.5)	0
Rash	16(11.0)	1(0.7)	4(3.1)	0
Upper respiratory tract infection	15(10.3)	0	8(6.2)	0
Hyperglycemia	14(9.7)	2(1.4)	13(10.0)	0
Hyperlipidemia	10(6.9)	1(0.7)	14(10.8)	0
Weight increased	9(6.2)	4(2.8)	16(12.3)	0

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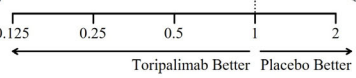
eFigure 1. Subgroup analysis of event-free survival (EFS)

Subgroup	Number of Events / Patients: Toripalimab	Placebo	Hazard Ratio (95% CI)	Subgroup P value	Interaction P value
Overall	47 / 202	97 / 202	0.40 (0.28, 0.57)	<0.001	
Age					0.57
< 65	33 / 140	66 / 138	0.41 (0.27, 0.62)	<0.001	
≥ 65	14 / 62	31 / 64	0.34 (0.18, 0.63)	0.005	
Sex					0.54
Male	42 / 181	91 / 189	0.38 (0.26, 0.54)	<0.001	
Female	5 / 21	6 / 13	0.54 (0.15, 1.80)	0.30	
Smoking					0.48
Yes (Including smoker and former)	39 / 174	88 / 181	0.37 (0.25, 0.54)	<0.001	
No	8 / 28	9 / 21	0.52 (0.19, 1.36)	0.17	
ECOG					0.61
0	20 / 70	38 / 73	0.44 (0.25, 0.74)	0.002	
1	27 / 132	59 / 129	0.36 (0.23, 0.57)	<0.001	
Histological type					0.29
Non squamous cell carcinoma	12 / 45	21 / 45	0.54 (0.26, 1.08)	0.08	
Squamous cell carcinoma	35 / 157	76 / 157	0.35 (0.23, 0.52)	<0.001	
PD-L1 expression					0.61
Not evaluable	4 / 18	7 / 16	0.44 (0.11, 1.45)	0.18	
Tumor cell < 1%	15 / 51	25 / 54	0.65 (0.33, 1.23)	0.19	
1% ≤ Tumor cell ≤ 49%	17 / 69	38 / 68	0.31 (0.17, 0.54)	<0.001	
Tumor cell ≥ 50%	11 / 64	27 / 64	0.31 (0.15, 0.60)	<0.001	
Disease stage					0.31
IIIA	34 / 136	65 / 136	0.44 (0.29, 0.66)	<0.001	
IIIB	13 / 65	31 / 64	0.30 (0.15, 0.56)	<0.001	



After a median follow up of 18.3 (12,7, 22.5) months, the number of events and patients, the hazardous ratios, P values and interaction P values are shown. EFS was defined as the time from randomization to the first documentation of disease progression leading to the inability to operate, postoperative progression, local or distant recurrence or death of any cause, whichever occurred first. PD-L1 positivity was defined as ⩾1% of tumor cells expressing PD-L1 by JS311 IHC staining in a central lab. All hazard ratios were computed using the Cox proportional hazards model. All P values were two-sided with no adjustment of multiplicity. The P values of testing the interaction of the subgroup variables with the treatment were computed using the Cox proportional hazards regression model with the treatment group, the subgroup variable and their interaction as the covariates.

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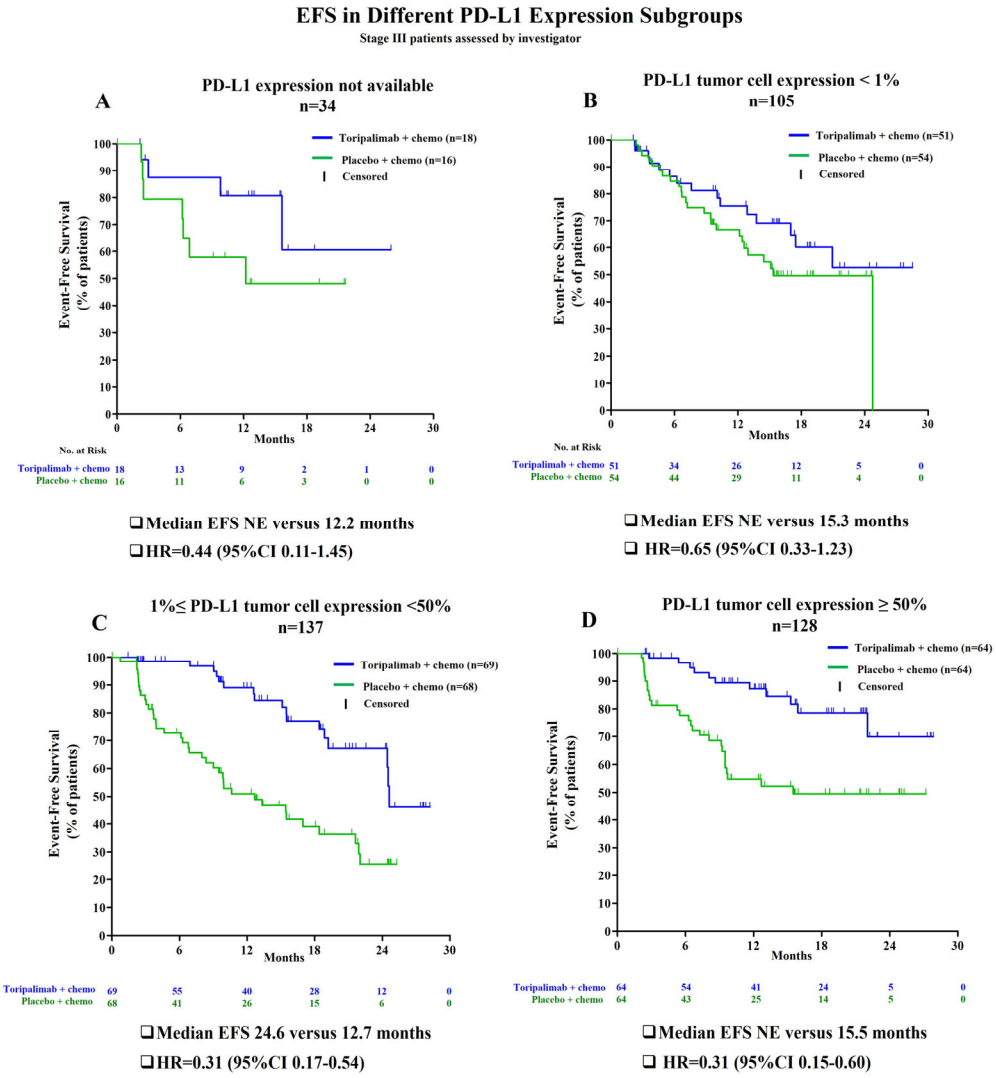

eFigure 2. Event-free survival (EFS) as assessed by investigators in PD-L1 expression subgroups

After a median follow up of 18.3 (12,7, 22.5) months, Kaplan-Meier-estimated EFS curves as assessed by the investigator compare toripalimab plus chemotherapy with placebo plus chemotherapy as perioperative treatment for patients with resectable stage III NSCLC in PD-L1 expression not available (Panel A), PD-L1 tumor cell expression <1% (Panel B), 1%≤PD-L1 tumor cell expression<50% (Panel C) and PD-L1 tumor cell expression ≥50% subgroups (Panel D). PD-L1 expression was evaluated by a validated JS311 IHC assay in a central lab. EFS was defined as the time from randomization to the first documentation of disease progression leading to the inability to operate, postoperative progression, local or distant recurrence or death of any cause, whichever occurred first. Censored patients are marked with " ❙ " in the graph, while the numbers of at-risk patients at indicated time points are below x-axis. The median EFS and stratified hazard ratios for EFS are shown. All hazard ratios were computed using the Cox proportional hazards model. NE, not estimable.

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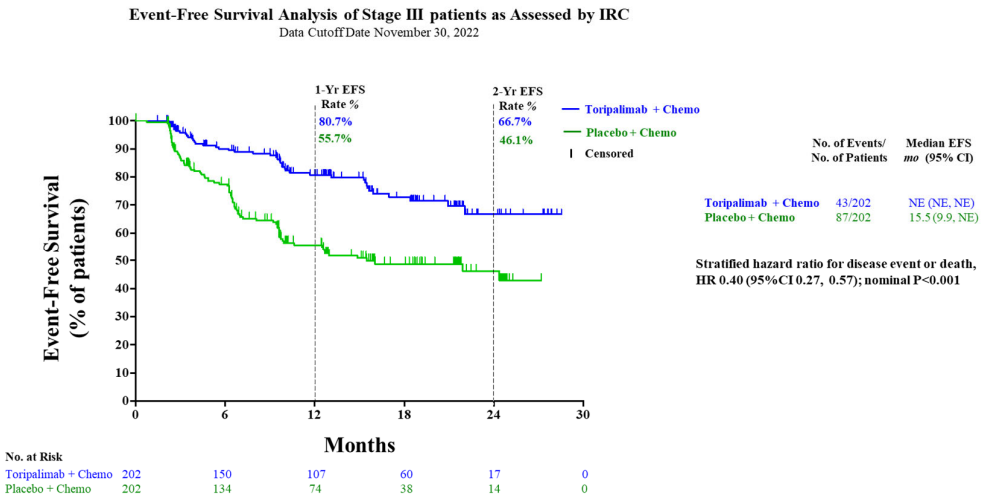

eFigure 3. Event-free survival (EFS) as assessed by independent review committee (IRC) in stage III NSCLC

After a median follow up of 18.3 (12,7, 22.5) months, Kaplan-Meier-estimated EFS curves as assessed by IRC compare toripalimab plus chemotherapy with placebo plus chemotherapy as perioperative treatment for patients with resectable stage III NSCLC. EFS was defined as the time from randomization to the first documentation of disease progression leading to the inability to operate, postoperative progression, local or distant recurrence or death of any cause, whichever occurred first. Censored patients are marked with " | " in the graph, while the numbers of at-risk patients at indicated time points are below x-axis. The number of events, median EFS, 1-year and 2-year EFS rates, and stratified hazard ratio for EFS are shown. The hazard ratio was computed using the Cox proportional hazards model. NE, not estimable.

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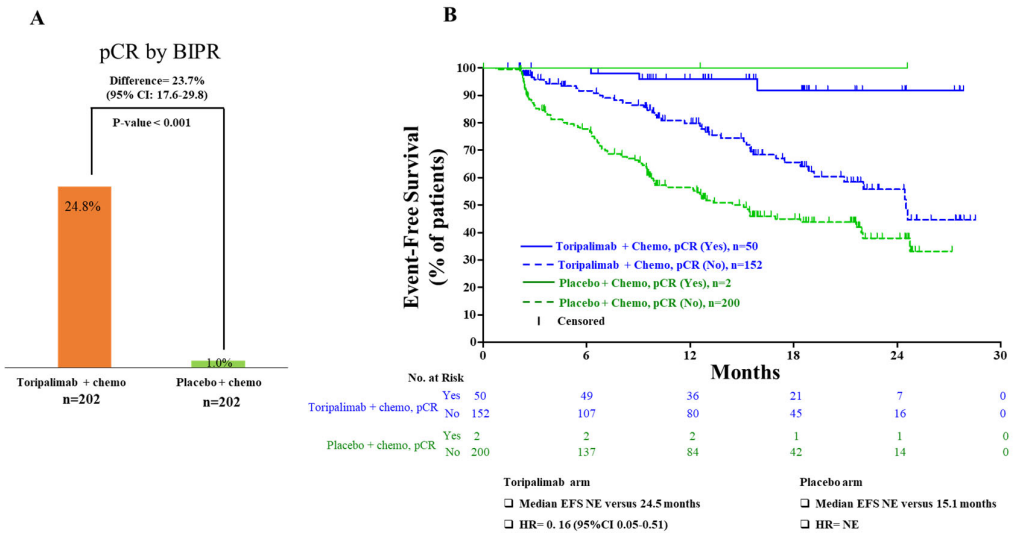

eFigure 4. Complete pathological response (pCR) as assessed by BIPR in stage III NSCLC patients and correlation of pCR with EFS

After a median follow up of 18.3 (12,7, 22.5) months, bar graph of pCR rates in toripalimab and placebo group are shown in Panel A. Kaplan-Meier estimates of EFS compare toripalimab plus chemotherapy with placebo plus chemotherapy group based on patients experiencing an pCR or not after neoadjuvant treatment (Panel B). pCR was defined as no residual tumor cells in the lungs and lymph nodes. EFS was defined as the time from randomization to the first documentation of disease progression leading to the inability to operate, postoperative progression, local or distant recurrence or death of any cause, whichever occurred first. Censored patients are marked with " ❙ " in the graph. The numbers of patients at risk at indicated time points are shown below the x-axis. The median EFS and stratified hazard ratios are at shown. All hazard ratios were computed using the Cox proportional hazards model. NE, not estimable.

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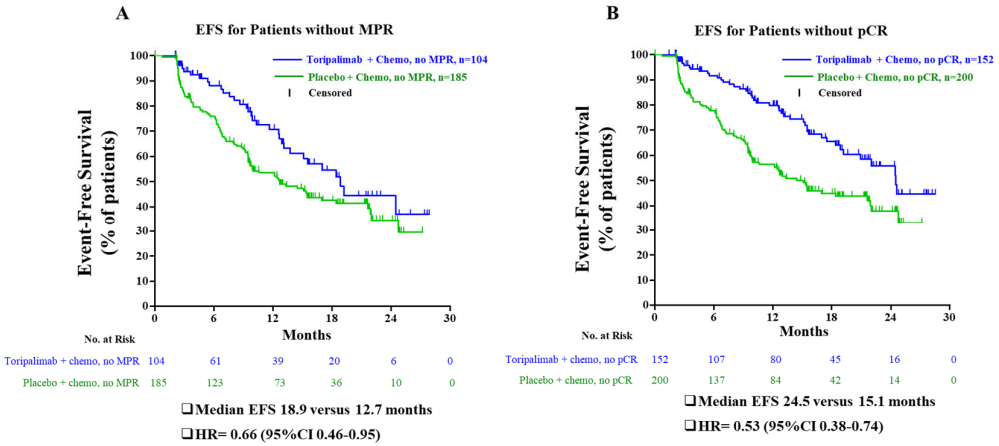

eFigure 5. Event-free survival (EFS) as assessed by investigators in patients without experiencing a major pathologic response (MPR) or complete pathological response (pCR)

After a median follow up of 18.3 (12,7, 22.5) months, Kaplan-Meier-estimated EFS curves as assessed by investigators compare toripalimab plus chemotherapy with placebo plus chemotherapy group as perioperative treatment for patients with resectable stage III NSCLC, who did not experience an MPR (Panel A) or pCR (Panel B) after neoadjuvant treatment. MPR was defined as 10% or less viable tumor cells in the tumor bed. pCR was defined as no residual tumor cells in the lungs and lymph nodes. EFS was defined as the time from randomization to the first documentation of disease progression leading to the inability to operate, postoperative progression, local or distant recurrence or death of any cause, whichever occurred first. Censored patients are marked with " | " in the graph, while the numbers of at-risk patients at indicated time points are below x-axis. The median EFS and stratified hazard ratios for EFS are shown. All hazard ratios were computed using the Cox proportional hazards model.

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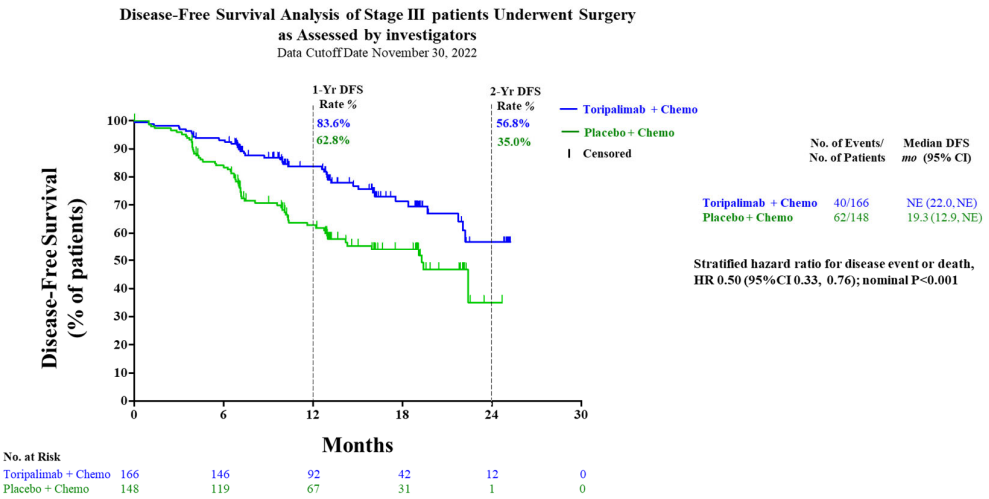

eFigure 6. Disease-free survival (DFS) as assessed by investigator in stage III NSCLC

After a median follow up of 18.3 (12,7, 22.5) months, Kaplan-Meier-estimated DFS curves as assessed by investigators compare toripalimab plus chemotherapy with placebo plus chemotherapy as perioperative treatment for patients with resectable stage III NSCLC, who had completed surgical resection. DFS was defined as the time from surgery to the first documentation of disease progression, local or distant recurrence, or death for any reason. Censored patients are marked with " ┃ " in the graph, while the numbers of at-risk patients at indicated time points are below x-axis. The number of events, median DFS, 1-year and 2-year DFS rates, and stratified hazard ratio for DFS are shown. All hazard ratios were computed using the Cox proportional hazards model. NE, not estimable.

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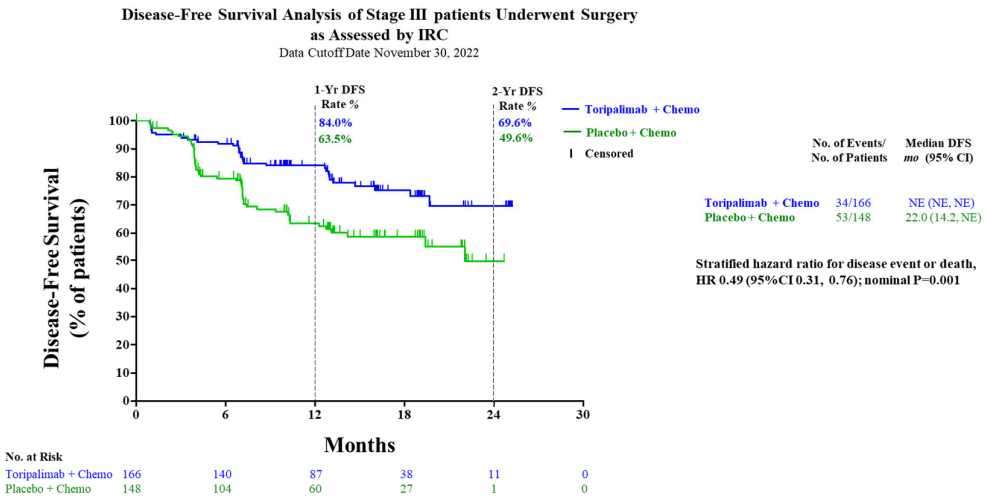

eFigure 7. Disease-free survival (DFS) as assessed by IRC in stage III NSCLC

After a median follow up of 18.3 (12,7, 22.5) months, Kaplan-Meier-estimated DFS curves as assessed by IRC compare toripalimab plus chemotherapy with placebo plus chemotherapy as perioperative treatment for patients with resectable stage III NSCLC, who had completed surgical resection. DFS was defined as the time from surgery to the first documentation of disease progression, local or distant recurrence, or death for any reason. Censored patients are marked with " | " in the graph, while the numbers of at-risk patients at indicated time points are below x-axis. The number of events, median DFS, 1-year and 2-year DFS rates, and stratified hazard ratio for DFS are shown. All hazard ratios were computed using the Cox proportional hazards model. NE, not estimable.

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